

CBT Session Transcript

Therapist: Good afternoon, Sarah. How have things been since our last session?

Patient: It's been a rough week. I keep thinking I'm failing at work. Every small mistake feels huge.

Therapist: Can you walk me through a recent situation where you felt that way?

Patient: Yesterday my supervisor corrected part of a report I submitted. After that, I couldn't stop thinking, "I'm terrible at my job."

INSERT QUESTION ABOUT CATASTROPHIC THINKING AND EFFECTIVE VS POOR CBT RESPONSE

Reasoning: Incorporating smaller questions throughout the activity will help to engage the learner through the activity so it doesn't become passive.

Question: How could you potentially respond to this client's reaction with an effective CBT response? Select all that apply.

Answer Options:

- a. "That's not true at all. You'll be fine if you think about the situation."
- b. "What suggests that a small correction makes you terrible at your job?"
- c. "I think you might be overreacting a bit to the situation."
- d. "What does the small correction mean to you?"

A & C Feedback (incorrect): These responses tend to minimize emotion and miss opportunities for cognitive restructuring.

B & D Feedback (correct): These responses promote cognitive flexibility and encourages evidence examination.

Therapist: When that thought came up — "I'm terrible at my job" — how intense was your anxiety from 0 to 10?

Patient: Probably an 8.

Therapist: And what emotions did you notice besides anxiety?

Patient: Shame. Embarrassment. I also felt hopeless

Question: How could you potentially respond to this client's reaction with an effective CBT response? Select all that apply.

Answer Options:

- a. You just need to think more positively (not correct- promotes toxic positivity)
- b. Things could be worse (not correct- invalidating)
- c. When people feel hopeless, it can become hard to notice exceptions or progress (correct- moves toward behavioral activation)
- d. What small change would feel realistic to you (correct- again moves toward behavioral activation)

Therapist: Let's slow the moment down. Your supervisor corrected part of your report. What evidence did your mind use to conclude that you're terrible at your job?

Patient: Well... the report had errors.

Therapist: Okay. Were there any facts that might not support the conclusion?

Patient: I guess the rest of the report was fine. And my supervisor said the corrections were "minor."

Therapist: That's important. Sometimes our thoughts jump from "I made a mistake" to "I'm incompetent." That's a cognitive distortion called overgeneralization. Does that fit what happened?

Patient: Yeah, definitely. I turned one mistake into proof that I'm a failure.

Therapist: If a coworker made the same mistake, would you think they were terrible at their job?

Patient: No. I'd probably think they were just human.

Question: How could you potentially respond to this client's reaction with an effective CBT response? Select all that apply.

Answer Options:

- a. Nobody's perfect (not correct, generic reassurance)
- b. You are being too hard on yourself (not correct, this does not challenge the rigid belief system)
- c. What makes it harder to apply the same standard to yourself? (correct, encourages balanced thinking)
- d. What might be a more balanced definition of success? (correct, explores cognitive rules and assumptions)

Patient: I feel like I'm supposed to be perfect all the time.

Therapist: That sounds like a very rigid rule. Where do you think that expectation comes from?

Patient: Growing up, mistakes weren't really tolerated in my house. My parents focused a lot on achievement.

Question: How could you potentially respond to this client's reaction with an effective CBT response? Select all that apply.

Answer Options:

That history can shape automatic beliefs. Let's try reframing the original thought. Instead of "I'm terrible at my job," what might be a more balanced statement? (correct- explores cognitive rules and assumptions)

- a.
- b. You are being too hard on yourself (not correct, this does not challenge the rigid belief system)
- c. What makes it harder to apply the same standard to yourself? (correct, encourages balanced thinking)
- d. What might be a more balanced definition of success? (correct, explores cognitive rules and assumptions)

Therapist: That history can shape automatic beliefs. Let's try reframing the original thought. Instead of "I'm terrible at my job," what might be a more balanced statement?

Patient: Maybe... "I made a mistake, but that doesn't mean I'm bad at my job."

Therapist: How believable does that feel right now, from 0 to 100 percent?

Patient: Around 60 percent.

Question: How could you potentially respond to this client's reaction with an effective CBT response? Select all that apply.

- a. That's a good start. What happens to your anxiety level when you say the balanced thought? (correct- encourages evidence examination)
- b. Just stop worrying about what people think (incorrect- dismissive and is advice giving without exploration)
- c. That's good enough (incorrect- dismissive without exploration)
- d. You should accept that you will have some anxiety about making a mistake (incorrect- dismissive)

Patient: It drops to maybe a 4.

Therapist: Great. That tells us the thought itself is influencing the emotional reaction. For homework this week, I'd like you to keep a thought record. When you notice strong self-criticism, write down:

1. The situation
2. The automatic thought
3. The emotion and intensity
4. Evidence for and against the thought
5. A more balanced replacement thought

Patient: Okay, I can do that.

Therapist: Before we end, what's one thing you're taking away from today's session?

Patient: I think I realized I treat my mistakes as proof that something is wrong with me, instead of seeing them as normal.

Question: How could you potentially respond to this client's reaction with an effective CBT response? Select all that apply.

- a. That's an important insight. We will continue to challenge those perfectionistic beliefs next session (correct- connects thoughts, emotions, and behaviors)
- b. What makes you think there is something wrong with you? (incorrect- invalidating)
- c. You have a lot going for you (incorrect, reassurance based)
- d. How would you evaluate this thought if a friend said it? (correct- encourages perspective taking)

Therapist: That's an important insight. We'll continue working on challenging those perfectionistic beliefs next session.

Patient: Thank you. This actually feels helpful.

Therapist: I'm glad to hear that. See you next week.

ENDING ASSESSMENT AFTER COMPLETING THE SCENARIO 5-10 QUESTIONS. These questions could be open-ended (will need to be manually graded) or you can create some multiple-choice questions. Make sure to provide feedback for each question.

Did the session follow a CBT structure? Why or why not? Provide two specific examples from the scenario.

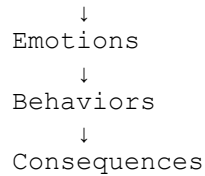
Yes

CBT model looks like this:

Situation

↓

Thoughts



For example:

- **Situation:** You have to give a presentation.
- **Thought:** "I'm going to embarrass myself."
- **Emotion:** Anxiety.
- **Behavior:** Avoid practicing or call in sick.
- **Consequence:** Temporary relief, but increased fear of presentations in the future.

CBT helps people:

1. Identify automatic thoughts.
2. Examine whether those thoughts are accurate or helpful.
3. Develop more balanced alternatives.
4. Practice new behaviors that support healthier outcomes.

A typical CBT session is also structured:

1. Review mood and recent events.
2. Set an agenda for the session.
3. Discuss a specific problem.
4. Identify thoughts, feelings, and behaviors involved.
5. Challenge unhelpful thinking patterns.
6. Develop coping strategies or behavioral experiments.
7. Assign homework/practice exercises.
8. Summarize key takeaways.

Another common CBT framework is the "ABC" model:

- **A** – Activating event
- **B** – Beliefs about the event
- **C** – Consequences (emotional and behavioral)

The idea is that our beliefs about a situation often have a stronger influence on our emotions and actions than the situation itself.

Which CBT interventions were used? Select all that apply.

- a. Provide an answer option? Identify thoughts, feelings, and behaviors involved
- b. Provide an answer option? Challenging of unhelpful thinking patterns
- c. Provide an answer option? Assign homework, practice exercises

- d. Provide an answer option ? Review mood and recent events